SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Nivolumab, ipilimumab, pemetrexed and cisplatin

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

First-line metastatic NSCLC · confirm pemetrexed-compatible histology · NCCP 00714 · version 4 · 42-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This treatment aims to control metastatic non-small-cell lung cancer, help symptoms and extend life. It is usually given without an expectation of cure. The team checks the cancer subtype, molecular results, your health and your preferences before recommending this particular chemotherapy backbone.

Schedule
The first cycle lasts 42 days: all four medicines on Day 1, then nivolumab, pemetrexed and cisplatin on Day 22. Chemotherapy ends after those two visits. From Cycle 2, nivolumab continues on Days 1 and 22 and ipilimumab on Day 1 of each 42-day cycle. Immune treatment continues while appropriate until the cancer progresses, side effects require stopping, or up to 24 months without progression. Plans can change after review.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Nivolumab
IMMUNOTHERAPY

- ☐ Immune inflammation in multiple organs
- ☐ Infusion reactions
- ☐ Delayed effects after treatment stops

Ipilimumab
IMMUNOTHERAPY

- ☐ Immune bowel, liver, skin and hormone inflammation
- ☐ Other serious organ inflammation
- ☐ Infusion reactions

Pemetrexed
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Mouth soreness, rash and fatigue
- ☐ Kidney toxicity

Cisplatin
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Nausea and vomiting
- ☐ Kidney, hearing and nerve effects
- ☐ Allergic reactions

Discuss treatment aims, alternatives, expected benefit and uncertainty, short-term and lasting harms, and effects on fertility, contraception, pregnancy, work and daily life. Your priorities and questions belong in the decision.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00714 v4.

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